

Patient ID : PA05

<i>CT Scan of Whole Abdomen</i>
--

Report:

Plain (only oral contrast for bowel opacification given) axial scans were taken in the region of interest. Intravenous contrast-enhanced axial scans were taken from the domes of the diaphragm to the pubic symphysis. Study reveals:

Liver: - Is enlarged in size and shows presence of multiple large well defined hypodense lesion scattered in both the lobes largest measuring~12x10x12 cm in size in inferior segment right lobe with thin peripheral enhancing wall. No evidence of intrahepatic biliary radical dilatation.

Gall bladder: - Is normal in diameter and wall thickness. No hyperdense intraluminal calculus is seen. No focal mass lesion is seen.

CBD: - is normal in diameter, with normal walls. No hyperdense intraluminal calculus is seen. **Portal vein is normal in calibre. SP axis** is patent.

Pancreas: - Is normal in size, attenuation and enhancement. Pancreatic duct is not dilated. No evidence of pancreatic calcification. No evidence of any peripancreatic inflammation or fluid collection.

Spleen: - Is mildly enlarged in size and normal in attenuation and enhancement. No focal space-occupying lesion is seen.

Kidneys: - Both kidneys are normal in size, position and attenuation. Kidney enhancement is normal and corticomedullary differentiation is maintained. No evidence of hydronephrosis, no calculi seen.

Bilateral **adrenal glands** are normal in size and attenuation.

Urinary Bladder: - Is well distended, normal in position. Bladder walls are not thickened, no mass lesion is seen. No intraluminal calculus is seen.

Uterus: Atrophied. No focal mass lesion is seen.

Bilateral adnexal region is seen normally. No focal mass lesion is seen.

Aorta and IVC are normal. No evidence of retroperitoneal or mesenteric lymphadenopathy. No evidence of ascites.

No abnormal bowel wall thickening or mass lesion is seen. No abnormally dilated bowel loops is seen.

Impression: Hepatomegaly with multiple large abscesses largest in inferior segment right lobe.

Mild splenomegaly.

Advise: Clinical Correlation